SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Triptorelin 11.25 mg: hormone injection every 12 weeks

Locally advanced or metastatic hormone-dependent prostate cancer · NCCP 00480 · version 3 · 84-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Schedule
Triptorelin 11.25 mg is given every 12 weeks (84 days). The usual NCCP route is into a muscle; the source also permits injection under the skin for this formulation.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Triptorelin 11.25 mg
GNRH AGONIST

- ☐ Hot flushes, sweating, fatigue and changes in sleep or mood
- ☐ Reduced sex drive, erection difficulties and reduced fertility
- ☐ Loss of bone strength, fracture risk, muscle loss and changes in weight
- ☐ Changes in glucose, cholesterol and cardiovascular risk
- ☐ Heart-rhythm changes; review other medicines that affect the QT interval
- ☐ Initial flare may worsen pain or urinary symptoms
- ☐ Injection-site pain or reactions
- ☐ Depression, allergic reactions and rare seizures or pituitary problems

Discuss treatment aim, duration, benefits and uncertainty, alternatives, sexual health and fertility, hot flushes, fatigue, mood, bone and muscle effects, metabolic/cardiovascular risks, interactions, formulation-specific risks, tests and urgent contacts. The team assesses temporary anti-androgen protection against the initial hormone flare. Report worsening pain or trouble passing urine promptly. New weakness or changes in bladder/bowel control require emergency assessment. Keep the written injection dates even if you feel well.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00480 v3.

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Developed by Dr Paul O'Brien